THEMES
determined from
interview insights

Themes

- Dosage administration and management, tracking
- symptom tracking
- big data picture tracking (over years, over medications, over fluctuations, over multiple datasets) analytics on projections
- communication - explaining it to other people, doctors, family members?
- food and other aspects of health impacting the symptoms/ illnesss/ allergic responses/ flareups
- general well being tracking that are non-medication related (phy/mental/other attributes/food/)
- education- not just for self, for peers/ family/ friends
- future repercussions of the medication - also the willingness to being informed
- community around it - specifically in India to the chronic illness (reduce loneliness + emotional support + networking/ opportunities + access)
- centralized report repository for comparision
- positive reaffirmation of things well-done

daily/weekly/monthly task sheet, reminder, nudge(r), scheduler eg:paintings/stretching/appointments/medication related

having peer/family/friend etc. to share certain emotional/physical data metrics on how you are doing

leveraging the pain-scale

- How might we simplify and make it easier for the patient to stay on top of the medication routine and adherence, and the dynamic changes in their dosage, frequency, other parameters as prescribed
- How might we simplify staying on track with the fluctations in symptoms to keep a record of the illness for the patient, doctor and the caregiver
- How might we create multiple visualizations of the gathered data - dosage/ symptoms/ mental and physical wellbeing/ food/ environment specially over the course of a timeframe
- How might we determine patterns, causation/ correlation between the symptoms of the patient during the fluctuation and change in medication (dosage/frequency) / other parameters (mental/physical well-being) -> future north-start: projections
- How might we empower the patient with projections using data analytics based on exisiting health and medication patterns
- How might we make subjective experience of chronic symptoms into more quantifable metric so that it is easily communicated to the doctors
- How might we sensitize and create awareness on chronic illnesses for peers, caregivers, patients, etc. and enable/empower them to support patient's betterment
- How might we track and map triggers to the illness/allergic responses, sensitivities, food maps, and degree of intensity of the flareups
- How might we enable patients to create and maintain a schedule of activities that benefit them - to serve as reminders/nudges/check-ins, and track progress over time.
- How might we enable patients to track other non-medicine related parameters, eg, sleep quality, food intake, mental health, physical health, and find corelations with other sets of data
- How might we help patients and caregivers, peers, etc educate themselves on specific chronic illnesses, their common experiences, what to expect, so as to help them understand the nature of the illness and also enable caregivers and peers understand, empathise and be better equipped to support patients.
- How might we create a community for patients to help tackle loneliness, and also provide a network for patients to find illness/disability friendly support and opportunities
- How might we make the data gathered accessible for visualization/comparison with respect to any parameter the user wishes, to view the progression of their illness.
- How might we make the process of tracking and staying on top of the illness a more positive experience for the patient, less of a chore.

pain scale + quantitatively share medication sympotions/impacts etc. understandable to layman

Yeah I've always had a hard time putting it in a number. Have come up with a more subjective system based on my normal activities during the day. Here, normal activities is skewed to represent activities during chronic illness, not the routine lifestyle before illness. So you can figure out your normal activity/routine level

1 is can do normalish activities without painkiller, but require rest.

2-3 is noticing that I'm avoiding certain activities like stairs and cooking and typing too much, but can manage without painkiller.

4-5 is thinking of taking painkiller but can just about carry on with basic tasks, need more rest, may or may not take a pill.

6-7 is not able to do many basic tasks either, will take painkiller, can do some work, but cautiously. Feeling quite drained by the effort going on.

8-9 is pain is absolutely front and center, not able to think about much else, despite painkiller. Using distractions to cope.

10 is immobilized with pain despite painkiller, distractions not helping either.